

Policy on High-Risk Patients — Identification and Management

«Hospital Name»

Document control

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Date created	_____	Date of implementation	_____
Prepared by	Nursing In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

Patients at risk of falls, pressure ulcers, deep vein thrombosis, or who are otherwise vulnerable, are identified using validated tools and protected by required precautions, and restraints are used only under a documented order with ongoing review.

1. Purpose

This policy explains how «Hospital Name» identifies vulnerable patients, patients at risk of falls, pressure ulcers, and deep vein thrombosis, manages them under written guidance, and uses restraints — physical or chemical — only when authorised, monitored, and documented.

This policy does not cover general patient assessment or general informed consent procedures in detail — those are covered in other hospital policies.

2. Scope

This policy applies to all clinical and nursing staff involved in identifying and managing high-risk patients at «Hospital Name».

3. Policy standards

«Hospital Name» identifies vulnerable patients, patients at risk of falls, pressure ulcers, and deep vein thrombosis, using validated tools and its own defined guidance, monitors

vulnerable patients at least twice a day, and uses restraints only under a documented clinical order with ongoing review and required monitoring.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

- Do not leave a patient identified as vulnerable without the precautions, monitoring, and consent process the written guidance requires, including monitoring at least twice a day.
- Do not assess fall risk, pressure-ulcer risk, or DVT risk with anything other than a validated tool, and do not leave a patient found at risk without management under written guidance.
- Do not leave the pressure-ulcer risk assessment covering only new ulcers — it must also cover worsening of existing ones.
- Do not apply or continue a restraint — physical or chemical — without a documented clinical order, without ongoing review, and without recording the reason and the time frame of use in the medical record.
- Do not skip the required elements of restraint guidance: who can authorise it, how often the patient is monitored, how long an order remains valid, and how the need for it is regularly reassessed.

5. What we do

5.1 Identify and manage vulnerable patients

Written guidance for identifying and managing vulnerable patients is developed in line with statutory requirements and national and international guidelines. It covers groups such as the elderly, children, differently-abled or mentally challenged patients, patients at risk of suicide or self-harm, comatose or critically ill patients, patients under sedation or anaesthesia, pregnant women, patients on dialysis, patients receiving chemotherapy, and immunosuppressed patients — these are examples, not the complete list; «Hospital Name» maintains its own comprehensive list.

The guidance states who is responsible for identifying these patients, how their risk is managed, and how they are monitored — at least twice a day. It also covers how informed consent is obtained from a vulnerable patient, and from the family or legal representative when the patient can't make an independent decision. Care for vulnerable patients follows this written guidance.

5.2 Identify and manage patients at risk of falls

A validated tool is used to assess fall risk. Patients found to be at risk are managed according to written guidance — universal fall precautions are a useful reference point.

5.3 Identify and manage patients at risk of pressure ulcers

A validated tool is used to assess the risk of developing pressure ulcers and of existing ones getting worse — for example, the Braden scale, or the EPUAP/NPUAP staging systems for tracking worsening. Patients found at risk are managed according to written guidance.

5.4 Identify and manage patients at risk of deep vein thrombosis

A validated tool — for example, Well's criteria — is used to assess DVT risk. Patients found at risk are managed according to written guidance.

5.5 Identify and manage patients who need restraints

"Restraints" covers both physical and chemical restraints. Written guidance identifies which patients need restraints and how they're cared for. «Hospital Name» minimises restraint use wherever possible, with patient safety in mind. The guidance covers situations where restraints could be used, who can authorise them, how often a restrained patient is monitored, and how long a restraint order stays valid. The need for restraint is regularly reassessed, and the least invasive option is chosen when one is needed. The rationale is explained to the family, with consent obtained where statutory requirements call for it. Restrained patients are monitored more frequently than usual, watching for complications the restraint itself could cause — for example, neurovascular deficit. When restraints are used, the medical record documents the reason for using them and the time frame during which they were used.

6. Stop-work authority

Do not leave a patient identified as vulnerable, at fall risk, at pressure-ulcer risk, at DVT risk, or needing restraints without the precautions and monitoring required by the organisation's written guidance.

Do not apply or continue restraints without a documented clinical order and ongoing review.

Stop-work applies to missing precautions or unauthorised restraint — not to emergency physical intervention needed to prevent immediate harm, which is followed at once by documentation and medical review.

The person who stops tells the treating doctor and the Ward/ICU In-Charge the same shift. Refusing to leave high-risk patients unprotected is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that this policy is followed and appropriately resourced; receives stop-work notifications the same shift.
Nursing In-Charge / Ward In-Charge	Ensures written guidance for all five high-

	risk categories is in place, current, and followed; ensures monitoring records are complete; receives stop-work notifications the same shift.
Treating doctors	Initiate validated-tool assessments; issue and review restraint orders; respond to stop-work notifications the same shift; document clinical orders and the reason and time frame for any restraint use in the medical record.
Nursing staff	Carry out validated-tool assessments; apply required precautions and monitoring (at least twice a day for vulnerable patients); monitor restrained patients more frequently; document as required; exercise stop-work authority when precautions are missing or restraints are unauthorised.
Quality Coordinator	Audits this policy quarterly; holds training acknowledgements.

8. Quality monitoring

Monitoring area	What is monitored
Vulnerable patient identification	Vulnerable patients identified using written guidance; monitoring conducted at least twice a day; consent process for incapable patients documented
Fall risk assessment	Validated tool used for every fall-risk assessment; patients at risk managed per written guidance; fall-precaution compliance monitored
Pressure ulcer risk assessment	Validated tool used; assessment covers both developing and worsening pressure ulcers — both scenarios; patients at risk managed per written guidance
DVT risk assessment	Validated tool used for DVT risk assessment; patients at risk managed per written guidance
Restraint use	Restraints (physical and chemical) applied and continued only under a documented clinical order; need reassessed regularly; all eight guidance elements present in written guidance; medical record documents reason and time frame for every restraint episode — both elements required
Stop-work events	Stop-work events logged with action taken and outcome; reviewed at next audit cycle

9. Training and staff acknowledgement

All clinical and nursing staff shall be familiar with the requirements of this policy, including the five high-risk categories, the validated-tool requirement for fall, pressure-ulcer, and DVT risk assessment, the exact monitoring frequency for vulnerable patients (at least twice a day), the pressure-ulcer dual scope (developing and worsening), the eight restraint guidance elements, the two mandatory medical record documentation requirements for restraints (reason and time frame), and the stop-work authority and escalation pathway.

I have read the Policy on High-Risk Patients — Identification and Management of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to all treating doctors, nursing and clinical staff involved in patient care at «Hospital Name».

11. Abbreviations

Abbreviation	Meaning
COP	Care of Patients (NABH Hospitals chapter)
DVT	Deep Vein Thrombosis
EPUAP	European Pressure Ulcer Advisory Panel
NABH	National Accreditation Board for Hospitals and Healthcare Providers
NPUAP	National Pressure Ulcer Advisory Panel

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
COP.16.a	CORE*	Section 5.1 addresses written guidance for vulnerable patients developed in line with statutory requirements and national/international guidelines, covering identification criteria, responsible persons, risk

		management, and monitoring at least twice a day, and the consent process for capable and incapable patients. CORE — non-compliance not acceptable. Stop-work authority (Section 6) also applies to this element.
COP.16.b	CORE*	Section 5.2 addresses validated-tool assessment for fall risk and management of patients found at risk under written guidance. A validated tool is required — a non-validated tool does not satisfy this OE. CORE. Stop-work authority (Section 6) also applies to this element.
COP.16.c	CORE*	Section 5.3 addresses validated-tool assessment for pressure-ulcer risk, covering both the risk of developing pressure ulcers and of existing ones getting worse — both scenarios are mandatory scope and are kept separate. Patients at risk managed per written guidance. CORE. Stop-work authority (Section 6) also applies to this element.
COP.16.d	CORE*	Section 5.4 addresses validated-tool assessment for DVT risk and management of patients found at risk under written guidance. CORE. Stop-work authority (Section 6) also applies to this element.
COP.16.e	Commitment*	Section 5.5 addresses restraints (physical and chemical): written guidance covers eight mandatory elements (identification criteria, care provision, situations for use, who can authorise, monitoring frequency, order validity,

		reassessment process, least-invasive selection); restraint minimisation; family explanation and consent where statutory requirements call for it; more-frequent monitoring for complications; medical record documentation of reason and time frame for every episode — both elements required. Stop-work authority covers missing precautions and unauthorised restraint.
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13. Required Records/Evidence Checklist

Vulnerable patients (COP.16.a)

- Written guidance for vulnerable patient identification and management, developed in line with statutory requirements and national/international guidelines.
- Monitoring records showing vulnerable patients assessed at least twice a day.
- Consent records: capable vulnerable patient's own consent, and family or legal representative consent for incapable patients.

Fall risk (COP.16.b)

- Validated fall-risk tool used for each assessment — tool identified and documented.
- Management records for patients found at fall risk, in line with written guidance.

Pressure ulcer risk (COP.16.c)

- Validated pressure-ulcer risk tool used for each assessment — covering both the risk of developing new pressure ulcers and the worsening of existing ones; both scenarios addressed.
- Management records for patients found at pressure-ulcer risk.

DVT risk (COP.16.d)

- Validated DVT risk tool used for each assessment.
- Management records for patients found at DVT risk.

Restraints (COP.16.e)

- Written guidance covering all eight mandatory elements: identification criteria, care provision, situations for use, authorisation, monitoring frequency, order validity, reassessment process, and least-invasive selection.

- Clinical orders for each restraint episode — documented before or at initiation; review records on file.
- Medical record entries for each restraint episode: reason for use and time frame of use — both elements required for every episode.
- Records of family explanation and consent where statutory requirements call for it.

Stop-work

- Stop-work events logged, with action taken and outcome documented.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. COP.16.

Disclaimer

This policy reorganises the supplied COP.16 objective elements and approved plain-language content into policy format. The stop-work text — covering all five high-risk categories (vulnerable, fall, pressure-ulcer, DVT, restraints) — is reproduced exactly as supplied, with role names in plain text (treating doctor; Ward/ICU In-Charge) and no guillemets. The exact monitoring frequency for vulnerable patients (at least twice a day) is preserved verbatim. The validated-tool requirement for fall, pressure-ulcer, and DVT risk is stated as mandatory — a non-validated tool does not satisfy the requirement. The pressure-ulcer OE scope covers both developing and worsening pressure ulcers — both scenarios are preserved and kept separate, and the Non-negotiable rules section states this as a distinct requirement. The eight restraint guidance elements and the two mandatory medical record documentation requirements (reason and time frame) are all preserved exactly.